



INTRACTABLE DEPRESSION AND MALINGERING

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1. Overview of Depression
2. Treatment Resistant Depression
3. Adjustment Disorder
4. Malingering
5. Questions

DEPRESSION – CORE SYMPTOMS

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LOW: mood, drive, energy, motivation, interest, enjoyment, concentration, activity, decisiveness

Thoughts:

Worthless
Loss of confidence and self-esteem
Guilt
Helpless/hopeless
Life not worth living/suicidal

Biological:

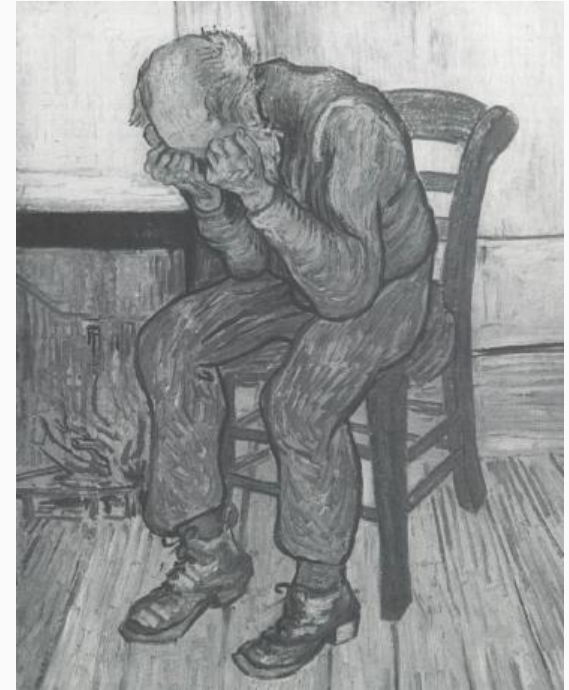
Poor sleep
Early morning waking
Poor appetite
Weight loss
Low sex drive
Mood lowest in morning

Psychotic Symptoms:

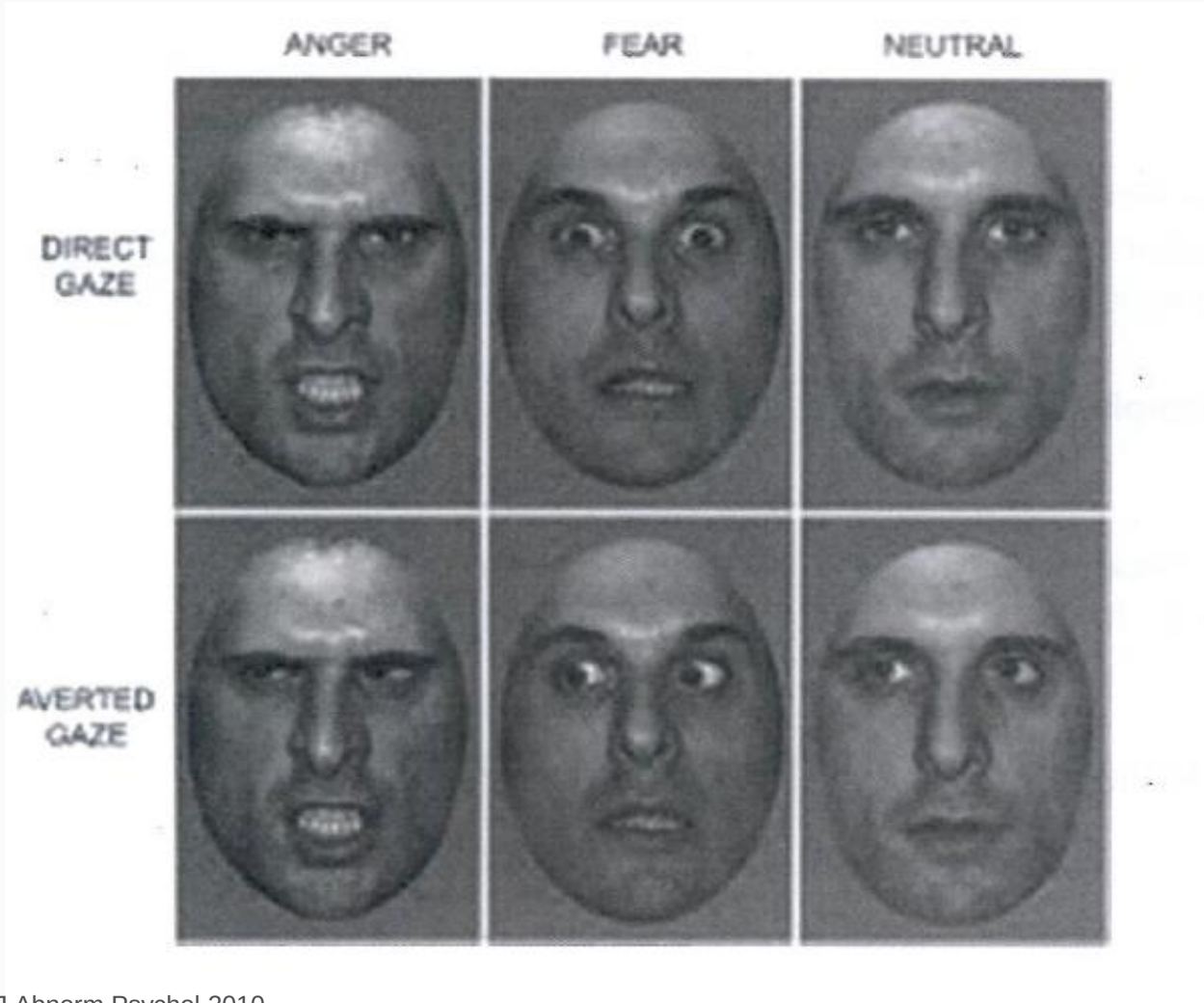
Delusions of e.g. poverty
Responsibility
Hallucinations – derogatory or accusing voices

DEPRESSION – SIGNS

Appearance:	Self-neglect, dejected
Behaviour:	Agitated/unresponsive
	Tearful
	Avoid eye-contact
	Hunched posture
Speech:	Slow
	Sparse
	Monotonous
	No spontaneous talk



EMOTIONAL RECOGNITION TESTS



Ref: Gollan et al J Abnorm Psychol 2010

UNTREATED DEPRESSION

Duration of Untreated Depression

USUALLY 6-18 MONTHS

Treatment-Resistant Depression

Failure to respond to:

max BNF dose

for at least 4 weeks of

two antidepressants or

one antidepressant and ECT



Causes of Apparent Treatment Failure

Incorrect diagnosis, i.e. not depression

Inadequate treatment

Poor compliance

Physical co morbidity

Psychiatric co morbidity, i.e. Drug/alcohol abuse, Personality disorder

NB undiagnosed bipolar disorder or a long period of untreated depression increase risk of treatment-resistance

Subjects with long histories of recurrent depression

2876 out patients with major depression

CITALOPRAM → 30 % remitted

SWITCH or AUGMENT → 25% remitted

SWITCH or AUGMENT → 12-20% remitted

MAOI or MTZ + VLFX → 7-10% remitted



Adjustment Disorder is a disputed diagnosis because of the blurred distinction between the varied manifestations of Adjustment Disorder and normal adaptive reactions (Casey, Dowrick and Wilkinson, 2001). These authors point out that "transient depressive responses to stressful events are increasingly regarded as illness requiring specific intervention" (p 480).



Diagnostic Criteria

- A. The development of emotional or behavioural symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).
- B. These symptoms or behaviours are clinically significant, as evidenced by one or both of the following:
 - 1. Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.
 - 2. Significant impairment in social, occupational, or other important areas of functioning.
- C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a pre-existing mental disorder.
- D. The symptoms do not represent normal bereavement.
- E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

BUT

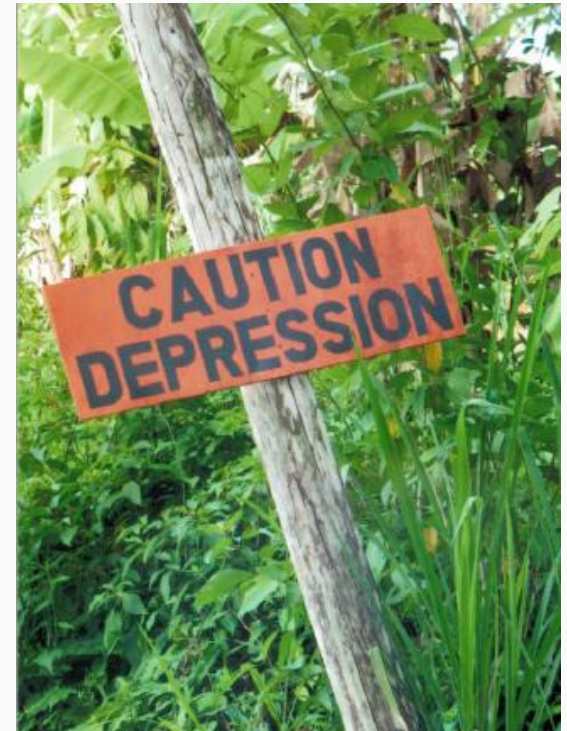
If the stressor or its consequences persist, the adjustment disorder may also continue to be present and become the persistent form.

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Malingering is a **behaviour** not a diagnosis or an illness

The presenting complaints and/or disability can be

- invented**
- exaggerated** or
- transferred** from an earlier injury or episode.



**RUTH HAS NEVER PLAYED AT THE O2,
SHE HASN'T GOT ANY GRAMMYS...**

**SHE HAS BEEN TREATED FOR
STRESS AT THE PRIORY.**

Ruth is 25 and a web designer from London;
she was helped by the Priory when the stress
of getting into severe debt became too much.

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PRIORY



MALINGERING AND FEIGNED SICKNESS

WITH NOTES ON THE WORKMEN'S COMPENSATION
ACT, 1906, AND COMPENSATION FOR INJURY,
INCLUDING THE LEADING CASES THEREON

BY

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WATER BOARD; CONSULTING MEDICAL EXAMINER TO THE SHIPPING FEDERATION;
MEDICAL EXAMINER TO THE SUN INSURANCE OFFICE, AND OTHER
ACCIDENT OFFICES; LATE HOME OFFICE MEDICAL REFEREE
WORKMEN'S COMPENSATION ACT

ILLUSTRATED



SECOND EDITION, REVISED AND ENLARGED

LONDON
EDWARD ARNOLD

1917
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THIS BOOK IS DEDICATED
TO MY FRIEND
THE BRITISH WORKMAN
TO WHOM I OWE MUCH

It is a mistake to think that all malingering is the outcome of deliberate wickedness. Because a man does not return to work as soon as one thinks he ought, it is harsh to assume that he is a shammer, and should be branded as a wilful malingerer. Such a view is not only unjust, but demonstrates a poor knowledge of human nature. Great allowance has to be made for the personal equation. Moral responsibility, even amongst the highly educated, is a variable quantity; indeed, it varies almost as much with different individuals as do the features. We cannot always fully appreciate the mental processes taking place in each individual mind, and, as long as unregenerate human nature is being dealt, with, so long are we bound **to weigh all the circumstances of each case**, if we wish to be fair.....

It is therefore of the first importance that a return to occupation, however light, should be recommended at the **earliest possible** time consistent with the nature of the ailment.

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“The essential feature of malingering is the intentional production of false or **grossly exaggerated** physical or **psychological symptoms** motivated by external incentives, e.g. avoiding military duty, **avoiding work, obtaining financial compensation** evading criminal prosecution or obtaining drugs”.

NOTE – this is not a diagnosis recognised in either DSM-5 or ICD-10

THE INDEPENDENT

Widow Blames 'bullying' Bank.

Richard Milner, Sunday 9th October 1994,

The widow of a financier who died of a heart attack after a two-year Bank of England investigation of his company has protested to the Bank's Governor, Eddie George, about the 'bullying' tactics of his enforcement officers. She plans to take legal action against two of them.

Harry Reuben, 69; who was accused of unlicensed deposit taking, was hounded to death, says Sylvia Reuben in a letter to Mr George. 'he suffered an acute nervous breakdown as a direct result of the oppressive and bullying behaviour of your investigators. He was allowed no peace at home or in hospital.

'I do not see how any institution like the Bank can get away with killing people like this. I shall not rest until my husband's death has been avenged by seeing these (men) punished.'

The bank's enforcer in chief- Peter Filmer- hired Dr Maurice Lipsedge to check on 25th July if Mr Reuben was fit to plead. He said 'let me die' again and again. Dr Lipsedge was not impressed. 'It was a grotesque parody', he decided in a written report, 'he is malingering'.

For several weeks, Harry Reuben did not see this report. Within hours of reading it, according to his wife, he suffered a heart attack and died on 29 August.

'I understand the distress this must have caused you' Mr George wrote to Mrs Reuben. He added 'Prosecutions are only brought after careful consideration of the available evidence. I am satisfied that the bank conducted itself properly in this instance'.



THANK YOU FOR YOUR ATTENTION

ANY QUESTIONS?

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